PAGE CONTENT & DESIGN PLAN · VERSION 2 (CONSOLIDATED PAGES)

Kasliwal Joint Institute

PRECISION. COMPASSION. MOTION.

/robotic-joint-replacement
/knee
/hip
/arthroscopy
/about
/patient-guide

Routes · demo layouts · section-by-section draft copy · SEO · NMC (6 Oct 2026) compliance

Prepared October 2026 · Draft for Dr. Vishal Kasliwal's medical review

VERSION 2 · CONSOLIDATED PAGES

How this document works

The website is moving from about 30 separate service pages to **six consolidated pages**, as decided by the team. Each page covers one area in full (for example, everything about the knee on **/knee**) with a sticky in-page menu that jumps to each topic. This document gives, for every page: the route and SEO settings, a demo layout (wireframe), and section-by-section draft copy written to the NMC Ethical Advertising Guidelines (6 October 2026).

How to read each section

- **Blue-bordered boxes** contain draft copy the developer can use directly once Dr. Kasliwal has reviewed it.
- **Grey cards** give design and SEO instructions for the section.
- **Amber boxes** flag NMC compliance points or facts the client must confirm before publishing.
- **#anchor** next to each section title is the in-page link ID, e.g. /knee#partial-knee-replacement.

The six pages

#	Route	Covers	Replaces
1	/robotic-joint-replacement	VELYS™ technology, robotic vs conventional, who it suits, robotic options, revision surgery	Robotic hub, VELYS page, revision page
2	/knee	Knee pain, arthritis, non-surgical care, “Do I need a knee replacement?”, robotic TKR, partial knee, recovery, risks, cost & insurance	Knee hub, arthritis, TKR, UKR, do-I-need, cost pages
3	/hip	Hip pain, arthritis, AVN, hip replacement, recovery, hip fracture in the elderly	Hip hub, AVN, robotic hip pages
4	/arthroscopy	Keyhole surgery, ACL, meniscus, PCL & cartilage, kneecap instability, shoulder, fracture & trauma care	Knee arthroscopy, ACL, meniscus, PCL, shoulder, trauma pages
5	/about	Dr. Vishal Kasliwal, training, surgical philosophy, the hospital, care team, visit us	Dr. page, philosophy, hospital (Patient Stories removed)
6	/patient-guide	Surgery journey, preparing, hospital stay, recovery, outstation patients, FAQs	All Patient Guide sub-pages

Contact, Book Appointment, Knowledge Centre and the legal pages stay as they are. Patient Stories is removed from the site and the About menu under the NMC rules.

SEO rules for consolidated pages

- **One topic owner per keyword.** To stop pages competing with each other: /knee owns “knee replacement Nashik” and “robotic knee replacement Nashik”; /robotic-joint-replacement owns “robotic joint replacement Nashik” and “VELYS Nashik”; /hip owns hip terms; /arthroscopy owns ACL, meniscus and shoulder terms. Where two pages touch the same topic, one has the full content and the other has a 2–3 line summary with a link.
- **Every sub-topic is an H2 with its keyword** (e.g. “Partial Knee Replacement (UKR) in Nashik”) and a stable anchor ID, so Google can show “jump to” links and the menus can point to it.
- **Sticky in-page tabs** under the hero (horizontal scroll on mobile), highlighting the current section.
- **Server-rendered HTML.** All content and menu links must be in the initial HTML, not loaded only on click. Accordions can hide answers visually, but the text must be in the page source.
- **301 redirects** from every old route to its new page (table on the next page), and update the sitemap.xml.
- **Trade-off to know:** one long page ranks strongly for its main terms but less strongly for narrow ones (e.g. “ACL surgery Nashik”). After 3–6 months, use Search Console data to split any section with high search demand into its

own page.

Redirect map (old route ↠ new location)

Old route	301 redirect to	Menu link (anchor)
/robotic-joint-replacement-nashik	/robotic-joint-replacement	/robotic-joint-replacement
/velys-robotic-surgery	/robotic-joint-replacement	#how-velys-works
/revision-joint-replacement-nashik	/robotic-joint-replacement	#revision-joint-replacement
/robotic-knee-replacement-nashik	/knee	#robotic-total-knee-replacement
/robotic-partial-knee-replacement-nashik	/knee	#partial-knee-replacement
/knee-specialist-nashik · /knee-arthritis-treatment-nashik	/knee	#knee-arthritis
/do-i-need-knee-replacement	/knee	#do-i-need-knee-replacement
/robotic-knee-replacement-cost-nashik	/knee	#cost-and-insurance
/hip-specialist-nashik · /robotic-hip-replacement-nashik	/hip	#hip-replacement
/avn-hip-treatment-nashik	/hip	#avn-hip
/knee-arthroscopy-nashik	/arthroscopy	#what-is-arthroscopy
/acl-reconstruction-nashik	/arthroscopy	#acl-reconstruction
/meniscus-tear-treatment-nashik	/arthroscopy	#meniscus-tear
/pcl-cartilage-surgery-nashik	/arthroscopy	#pcl-and-cartilage
/shoulder-arthroscopy-nashik	/arthroscopy	#shoulder-arthroscopy
/orthopaedic-trauma-care-nashik	/arthroscopy	#fracture-and-trauma-care
/dr-vishal-kasliwal · /surgical-philosophy · /hospital	/about	#dr-vishal-kasliwal · #surgical-philosophy · #hospital
/patient-stories	/about	(removed from menu)
/patient-guide/*	/patient-guide	#journey · #preparing · #recovery · #outstation-patients · #faqs

Redirects can't include #anchors reliably

A server redirect drops the #fragment, so old URLs land at the top of the new page. That is fine for SEO. The menus should link directly to the anchors.

Elements repeated on every page

Element	Specification
Breadcrumb	Home › Knee (BreadcrumbList schema)
Hero	H1, one-line description, [Book a Consultation] + [Call] + [WhatsApp], real photo (no AI patients).
Doctor credential line	“Dr. Vishal Kasliwal · M.S. Orthopaedics · Reg. No. [MMC/NMR number]”. The registration number is required under the NMC guidelines.

Element	Specification
Sticky in-page tabs	One tab per H2 section; current tab highlighted.
Mid-page CTA band	After the main procedure section: “Not sure which option suits you? Book an assessment.”
FAQ accordion	8–12 questions with FAQPage schema; text present in the HTML.
Medical review box	“Medically reviewed by Dr. Vishal Kasliwal, M.S. Ortho · Last updated [date]”.
Disclaimer line	“This information is for education and does not replace a consultation. Treatment and recovery differ between patients.”
Closing CTA + footer	As on the homepage.

NMC rules applied to all copy in this document

- No “best”, “No. 1”, “only”, “first”, “painless”, “guaranteed” or success-rate claims.
- No testimonials, reviews, ratings, before/after photos or AI-generated patient images.
- Numbers (years, procedure counts) only if Dr. Kasliwal can show records.
- Benefits phrased as “may” or “studies show”, with honest limits; recovery given as typical ranges.
- No discounts, offers or promotional EMI messaging.

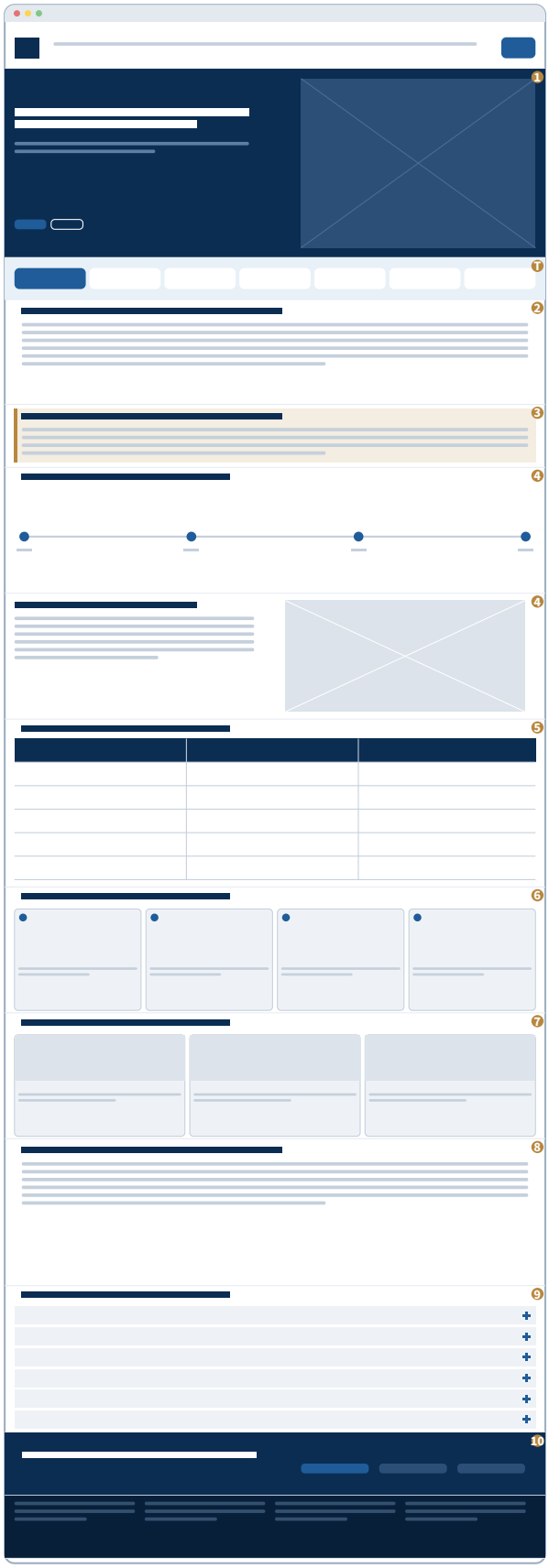
PAGE 1

Robotic Joint Replacement

Route	**/robotic-joint-replacement**
H1	**Robotic Joint Replacement in Nashik**
Title tag	Robotic Joint Replacement in Nashik | VELYS™ | Kasliwal Joint Institute
Meta description	How VELYS™ robotic-assisted knee replacement works, how it compares with conventional surgery, who it suits, and revision joint replacement in Nashik.
Primary keywords	robotic joint replacement Nashik · VELYS robotic surgery Nashik · robotic surgery Nashik
Secondary keywords	robotic vs conventional knee replacement · revision knee replacement Nashik · revision hip replacement Nashik · is robotic knee replacement worth it
Schema (JSON-LD)	MedicalWebPage · MedicalProcedure · Physician · MedicalClinic · FAQPage · BreadcrumbList
Target length	2,500–3,500 words
Replaces old routes	/robotic-joint-replacement-nashik, /velys-robotic-surgery, /revision-joint-replacement-nashik

Ownership

This page explains the **technology** and robotic surgery in general. The detailed clinical content for robotic total and partial knee replacement lives on **/knee**, and for hip replacement on **/hip**. This page summarises them and links there.



Demo layout (wireframe)

Numbers match the section numbers in the content pages that follow. T = sticky tabs.

1 Hero
H1, sub-line, Book / Call / WhatsApp, real photo of VELYS™ in the institute's OT, credential line.

T Sticky tabs
Overview · How VELYS works · Robotic vs conventional · Who it suits · Options · Revision · FAQs

2 What is robotic-assisted joint replacement?
Intro copy.

3 Surgeon-in-control callout
Sand box with gold border.

4 How VELYS™ works
Four numbered steps, then image + technology facts.

5 Robotic vs conventional
Evidence-based comparison table.

6 Who it may suit
Icon cards: suitable / may not need.

7 Robotic options
Three cards linking to /knee and /hip anchors.

8 Revision joint replacement
Full section with reasons for failure.

9 FAQs
Accordion, 8 questions.

10 Closing CTA

Section-by-section content

1 Hero

#top · First screen: tells patients and Google what the page is about.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

H1: Robotic Joint Replacement in Nashik

Robotic-assisted knee replacement with the VELYS™ system, planned around your anatomy and performed by fellowship-trained joint replacement surgeon Dr. Vishal Kasliwal at Kasliwal Joint Institute.

Buttons: [Book a Consultation] [Call +91 99223 35007] [WhatsApp]

Credential line: Dr. Vishal Kasliwal · M.S. Orthopaedics · Fellowship-trained in Joint Replacement & Arthroscopy · Reg. No. [to add]

Design	Navy hero, text left, real photo of Dr. Kasliwal at the VELYS™ console right. Mobile: photo above text.

NMC check

Phone number must be the confirmed primary number (see the earlier audit).

2 What is robotic-assisted joint replacement?

#overview · Plain-language definition.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Joint replacement replaces the worn surfaces of an arthritic joint with smooth metal and medical-grade plastic components. In robotic-assisted surgery the operation is the same, but the surgeon uses a computer and a robotic tool to measure your joint during surgery, plan the position of each implant and prepare the bone to that plan.

At Kasliwal Joint Institute, robotic-assisted knee replacement is performed with the VELYS™ Robotic-Assisted Solution. The aim is a joint replacement that is precisely positioned, balanced with your ligaments, and suited to your individual anatomy.

SEO	Include “robotic joint replacement in Nashik” naturally in the first 100 words.

3 The surgeon stays in control

#surgeon-in-control · Builds trust; answers the most common fear.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

The robot doesn't replace the surgeon

Robotic technology is a surgical tool. It does not move or operate on its own. Dr. Kasliwal examines you, decides whether surgery is needed, chooses the operation, creates the surgical plan and performs the procedure. The robotic system provides measurements and guidance that help him carry out that plan with precision and control.

Design	Sand callout with gold left border and a small portrait of Dr. Kasliwal.

4 How VELYS™ robotic-assisted knee replacement works

#how-velys-works · Step-by-step explanation; replaces the old /velys-robotic-surgery page.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

VELYS™ is an image-free robotic system made by DePuy Synthes (Johnson & Johnson MedTech). Image-free means you do not need a special CT or MRI scan for the robot before surgery; your routine X-rays are used for diagnosis and planning, and the system measures your knee directly during the operation. The unit attaches to the side of the operating table and uses an infrared camera to track the position of your knee in real time.

1. Plan

After the knee is exposed, Dr. Kasliwal maps key points on the thigh and shin bones. The system builds a model of your knee's alignment and calculates the implant size and position.

2. Balance

Your knee is moved through its range of motion. The system shows how tight or loose the ligaments are in different positions, so the plan can be adjusted to balance the soft tissues rather than relying on visual judgement alone.

3. Guide

The robotic tool helps prepare the bone surfaces according to the final plan, within boundaries the surgeon sets.

4. Verify

Before the final implants are fixed, alignment, stability and movement are checked again, and adjustments can be made if needed.

VELYS™ is used with the ATTUNE® Knee System implant.

Design	Four numbered step cards (as on the homepage: Plan · Guide · Balance · Verify), then an image + facts block: Image-free · Table-mounted · Real-time tracking · Surgeon-controlled. Short OT video by Dr. Kasliwal (Phase 2).
SEO	H2 includes “VELYS™ Robotic Knee Replacement”. Add VideoObject schema when the video is ready.

NMC check

- Confirm the institute uses the ATTUNE® Knee with VELYS™ and the exact step order Dr. Kasliwal follows (the homepage says Plan · Guide · Balance · Verify; manufacturer workflow balances before cutting). Use one order on both pages.
- Credit the trademark: VELYS™ and ATTUNE® are trademarks of DePuy Synthes; no manufacturer logos without permission.

5 Robotic vs conventional knee replacement

#robotic-vs-conventional · Honest, evidence-based comparison. This is the main NMC-sensitive section.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Both robotic-assisted and conventional knee replacement are well-established operations. The difference is how the surgeon measures, plans and prepares the bone.

	Conventional	Robotic-assisted (VELYS™)
The operation	Same implants and approach	Same implants and approach
Planning	Mechanical guides and the surgeon's experience	Measurements of your knee taken during surgery
Implant positioning	Accurate in experienced hands	Studies show more consistent positioning and fewer alignment outliers
Ligament balance	Judged by feel and spacers	Measured through the knee's range of motion
Pre-operative scan	Routine X-rays	Routine X-rays (no CT/MRI needed for VELYS™)
Long-term results	Decades of published data	Promising; long-term data still being collected
Cost	Lower	Higher, because of the technology

What the evidence says: published research on robotic knee replacement, including a 2026 scoping review of VELYS™, shows improved accuracy of implant positioning with a safety profile comparable to conventional surgery. Whether this leads to better long-term function or longer-lasting implants is still being studied. We will explain both options and help you decide which suits you.

SEO	Targets “robotic vs conventional knee replacement” and “is robotic knee replacement worth it”. Link the review in a references line.

NMC check

Do not claim faster recovery, less pain, longer implant life or better results as facts. The copy above stays within what the evidence supports.

6 Who is robotic-assisted surgery suitable for?

#who-is-it-for · Candidacy, with honest limits.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

It may be suitable if:

- You have knee arthritis that still limits walking, sleep or daily life after non-surgical treatment.
- X-rays and examination show joint damage that needs a replacement.
- You have a deformity such as bow legs or knock knees, where precise alignment and balance are especially important.
- You are medically fit for surgery and anaesthesia.

You may not need it, or may not be suitable, if:

- Your symptoms can still be managed with exercise, weight control, medicines or injections.
- You have an active infection, or medical conditions that make surgery unsafe until treated.
- Your knee problem is better treated another way, for example with arthroscopy or a partial replacement.

Robotic assistance is a way of performing the operation, not a reason to have it. The decision to operate is made on your symptoms, examination and X-rays.

Design	Two icon-card columns: green ticks (may suit) and neutral info icons (may not need).

7 Robotic joint replacement options

#options · Routes visitors to the detailed pages.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

- **Robotic Total Knee Replacement:** for arthritis affecting most of the knee. → /knee#robotic-total-knee-replacement
- **Partial Knee Replacement:** for arthritis limited to one part of the knee. → /knee#partial-knee-replacement
- **Hip Replacement:** technology-assisted total hip replacement. → /hip#hip-replacement

Design	Three cards with implant illustrations (not AI surgical scenes) and “Read more →”.

NMC check

- Partial knee: older Kasliwal sites mention the **Oxford** partial knee. VELYS™ is cleared for total knee replacement. Confirm whether partial knees are robotic-assisted at the institute. If not, call the card “Partial Knee Replacement” without “Robotic”.
- Hip: confirm whether hips use robotic assistance or a navigation system, and name it accurately.

8 Revision joint replacement

#revision-joint-replacement · Full content for knee and hip revisions; replaces the old revision page.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Most knee and hip replacements work well for many years. Sometimes a replacement becomes painful, loose or unstable and needs a second operation, called revision surgery. Revision is more complex than a first replacement and needs careful assessment and planning.

Why a joint replacement may need revision

- Infection around the implant
- Loosening of the implant from the bone
- Instability or repeated dislocation
- Wear of the plastic bearing surface
- A fracture of the bone around the implant
- Stiffness or persistent pain
- Bone loss around the implant

Signs that your replacement should be checked

- New or increasing pain, especially after a pain-free period
- Swelling, warmth, redness or wound discharge
- A feeling of the joint giving way or clicking
- Reduced movement or a change in leg length

How revision is assessed and planned

Assessment includes examination, X-rays and, where needed, blood tests and joint fluid tests to rule out infection. Revision may involve replacing one or all components, using special implants to manage bone loss, and sometimes treating infection in two stages. The plan, risks and expected recovery are discussed in detail before surgery.

If you have had a replacement elsewhere and have concerns, you can share your X-rays and reports for an assessment.

Design	Text section with two icon lists side by side; [Request a revision assessment] button opening the booking form with report upload.
SEO	H2: “Revision Knee & Hip Replacement in Nashik”.

NMC check
Revision is not described as robotic. Confirm whether Dr. Kasliwal regularly performs revisions before publishing.

9 FAQs

#faqs · Accordion with FAQPage schema.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Does the robot perform the operation?
No. Dr. Kasliwal performs the operation. The robotic system helps with measurement, planning and bone preparation, within limits he sets.

Do I need a CT scan before robotic knee replacement?
Not for VELYS™. It is an image-free system that measures your knee during surgery. Routine X-rays and blood tests are still needed.

Is robotic knee replacement safe?
Studies show a safety profile comparable to conventional knee replacement. All surgery has risks, which are explained before you decide.

Will I recover faster with robotic surgery?
Recovery depends mainly on your health, the operation and physiotherapy. Some studies report early benefits, but evidence is still developing, so we do not promise faster recovery.

Will a robotic knee last longer?
Better implant positioning may help, but long-term studies are still in progress.

Is robotic surgery more expensive?
Yes, the technology adds to the cost. See Cost & Insurance on the Knee page for what affects the total.

Is robotic knee replacement covered by insurance?
Many policies cover joint replacement, including robotic-assisted procedures, but coverage varies. Our team can help you check your policy.

Can both knees be replaced robotically?
Yes, either on the same day or in two stages. Which suits you depends on your health; Dr. Kasliwal will advise you.

10 Closing CTA

#book ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Considering joint replacement? Start with an assessment. Dr. Kasliwal will examine your joint, review your X-rays and explain your options, including whether surgery is needed at all.
[Book a Consultation] [Call] [WhatsApp]

PAGE 2

Knee

Route	**/knee**
H1	**Knee Treatment & Knee Replacement in Nashik**
Title tag	Knee Replacement in Nashik | Robotic & Partial Knee | Dr. Vishal Kasliwal
Meta description	Knee arthritis, robotic total knee replacement, partial knee replacement, recovery and costs in Nashik. Assessment by Dr. Vishal Kasliwal.
Primary keywords	knee replacement Nashik · robotic knee replacement Nashik · knee specialist Nashik · knee replacement surgeon Nashik
Secondary keywords	partial knee replacement Nashik · knee arthritis treatment Nashik · knee pain doctor Nashik · knee replacement cost Nashik · do I need knee replacement
Schema (JSON-LD)	MedicalWebPage · MedicalCondition (osteoarthritis of knee) · MedicalProcedure (TKR, UKR) · Physician · FAQPage · BreadcrumbList
Target length	4,000–5,500 words (the longest page)
Replaces old routes	/knee-specialist-nashik, /knee-arthritis-treatment-nashik, /robotic-knee-replacement-nashik, /robotic-partial-knee-replacement-nashik, /do-i-need-knee-replacement, /robotic-knee-replacement-cost-nashik



Demo layout (wireframe)

Numbers match the section numbers in the content pages that follow. T = sticky tabs.

1 Hero
H1, sub-line, buttons, real photo (consultation or OT).

T Sticky tabs
Knee pain · Arthritis · Do I need surgery? · Robotic TKR · Partial knee · Recovery · Cost · FAQs

2 Common knee problems
6 icon cards linking to sections or /arthroscopy.

3 Knee arthritis
Diagram of healthy vs arthritic knee + copy.

4 Non-surgical treatment
Step ladder graphic.

5 Do I need a knee replacement?
Interactive self-check.

6 Robotic total knee replacement
Image + full procedure content.

7 Partial knee replacement
Partial vs total comparison table.

8 Recovery timeline
Horizontal timeline (vertical on mobile).

9 Risks
Text + list.

10 Sports injuries & revision
Cards linking to /arthroscopy and /robotic-joint-replacement.

11 Cost & insurance
Cost-factor table + insurance copy.

12 FAQs
Accordion, 10 questions.

13 Closing CTA

Section-by-section content

1 Hero

#top ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

H1: Knee Treatment & Knee Replacement in Nashik

From knee pain and arthritis to robotic total and partial knee replacement: assessment and treatment by Dr. Vishal Kasliwal, joint replacement and arthroscopy surgeon at Kasliwal Joint Institute, Nashik.

[Book a Consultation] [Call] [WhatsApp] · Credential line with registration number

2 Common knee problems we treat

#knee-pain · Lets visitors find their problem quickly.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Knee pain has many causes. Getting the diagnosis right comes first, because the right treatment depends on it.

- **Knee arthritis**: wear of the joint cartilage, causing pain, stiffness and deformity. → #knee-arthritis
- **Ligament injuries (ACL, PCL)**: often from sports, falls or two-wheeler accidents. → /arthroscopy#acl-reconstruction
- **Meniscus tears**: catching, locking or swelling after a twist. → /arthroscopy#meniscus-tear
- **Cartilage damage**: localised damage to the joint surface. → /arthroscopy#pcl-and-cartilage
- **Kneecap instability**: the kneecap slipping out of place. → /arthroscopy#kneecap-instability
- **A painful or failed knee replacement** → /robotic-joint-replacement#revision-joint-replacement

Design	Six icon cards in a 3×2 grid (2×3 on mobile). Use line icons, not photos.

3 Knee arthritis

#knee-arthritis · Condition content; replaces the knee arthritis page.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Knee arthritis is the gradual wearing of the smooth cartilage that covers the ends of the bones. As the cartilage thins, the bones rub together, causing pain, swelling and stiffness. It is very common after the age of 50, and in India it often affects the inner side of the knee, leading to bow legs.

Types

- **Osteoarthritis**: the most common type, linked to age, weight, previous injury and family history.
- **Rheumatoid and other inflammatory arthritis**: the immune system attacks the joint lining, often in both knees.
- **Post-traumatic arthritis**: develops years after a fracture or ligament injury.

Common symptoms

- Pain on walking, climbing stairs or getting up from a chair
- Morning stiffness and swelling
- Difficulty sitting cross-legged or squatting
- Bow legs or knock knees that gradually increase
- Grinding or cracking sounds
- Pain at night that disturbs sleep

How it is diagnosed

Diagnosis is based on your symptoms, an examination of the knee and standing X-rays. A long-leg X-ray (scanogram) may be taken to measure alignment. MRI is needed only in selected cases.

Design	Split block: labelled diagram of a healthy vs arthritic knee (illustration) + copy.
SEO	H2: “Knee Arthritis Treatment in Nashik”.

4 Non-surgical treatment comes first

#non-surgical-treatment · Shows balanced, conservative-first care (strong for NMC and trust).

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Many people with knee arthritis manage well without surgery, especially in the early stages.

- **Exercise and physiotherapy**: strengthening the thigh muscles reduces load on the knee and is one of the most effective treatments.
- **Weight management**: every kilogram lost reduces the load on the knees when walking.
- **Activity changes**: reducing deep squatting, frequent stairs and floor sitting during flare-ups.
- **Medicines**: pain relievers and anti-inflammatory medicines for short periods, as prescribed.
- **Supports**: a walking stick in the opposite hand, cushioned footwear or a knee brace.
- **Injections**: steroid injections can relieve flare-ups for a period; evidence for other injections varies and is discussed individually.

Surgery is considered when these measures no longer control pain or allow you to live the way you need to.

Design	Ladder/steps graphic: Lifestyle → Physiotherapy → Medicines → Injections → Partial knee → Total knee.

5 Do I need a knee replacement?

#do-i-need-knee-replacement · Self-assessment; replaces the old page.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Ask yourself these questions:

- Does knee pain wake you at night or stop you from sleeping?
- Can you walk only a short distance before stopping?
- Is climbing stairs or getting out of a chair difficult?
- Is your knee becoming more bowed or knock-kneed?
- Do you depend on painkillers most days?
- Have physiotherapy, weight control and injections stopped helping?
- Is the knee limiting your work, travel, religious or social activities?

If you answered yes to several of these, it may be time for a specialist assessment. The decision for knee replacement is based on your symptoms, how much the knee limits your life, examination and X-rays, not on an X-ray alone. A replacement is a choice you make with your surgeon, when the benefits are likely to outweigh the risks.

Design	Interactive checklist: tick boxes; result shows “Consider a specialist assessment” with [Book] button. Never show a diagnosis or a score claiming the user needs surgery.
SEO	H2: “Do I Need a Knee Replacement?” (matches a common search).

NMC check

Must not frighten or pressure. Keep the result message neutral and educational.

6 Robotic total knee replacement (TKR)

#robotic-total-knee-replacement · Main procedure content for the page's top keyword.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

In a total knee replacement, the damaged surfaces of the thigh bone (femur) and shin bone (tibia) are replaced with metal components, with a medical-grade plastic insert between them. The back of the kneecap may also be resurfaced. At Kasliwal Joint Institute, total knee replacement is performed with VELYS™ robotic assistance, which helps plan implant position and ligament balance around your anatomy.

Step by step

- **Assessment**: examination, standing and long-leg X-rays, and a discussion of your goals.
- **Pre-operative tests**: blood tests, ECG and a fitness check with the physician and anaesthetist; diabetes and blood pressure are brought under control.
- **Anaesthesia**: usually spinal anaesthesia, often with a nerve block for pain relief after surgery.
- **Surgery**: the knee is exposed, measured with the robotic system, balanced, prepared and the implants fixed. It usually takes about 1.5 to 2 hours.
- **Early mobilisation**: standing and walking with support usually begin within a day of surgery, guided by physiotherapists.
- **Going home**: typically after a few days, once you can walk safely with support and manage stairs if needed.

Our surgical approach

Where appropriate, Dr. Kasliwal uses a muscle-sparing (subvastus) approach and plans alignment around your natural anatomy rather than one fixed target for every knee. Read more in Our Surgical Philosophy (/about#surgical-philosophy).

One knee or both?

If both knees are affected, they can be replaced on the same day or a few weeks apart. Same-day surgery means one hospital stay and one recovery, but it is a bigger operation; it is advised only if your health allows.

Design	Split: real OT/VELYS photo + copy, then a horizontal 6-step strip.
SEO	H2: “Robotic Total Knee Replacement in Nashik”. Primary keyword for the page.

NMC check

Confirm operation time, anaesthesia and length of stay with Dr. Kasliwal; these are typical figures, not promises.

7 Partial knee replacement (UKR)

#partial-knee-replacement · Replaces the old UKR page.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Not every arthritic knee needs a total knee replacement. When arthritis is limited to one part of the knee, usually the inner (medial) side, only that part can be replaced. This is called a partial or unicompartmental knee replacement.

Who may be suitable

- Arthritis confined to one compartment of the knee
- Intact, functioning ligaments, especially the ACL
- A deformity that can be corrected and a good range of movement
- No inflammatory arthritis

	Partial knee replacement	Total knee replacement
Part replaced	One compartment	Whole joint surface
Bone and ligaments	More of your own bone and ligaments kept	Some ligaments may be removed, depending on implant
Incision	Usually smaller	Standard
Early recovery	Often quicker	Predictable, usually longer
Suitability	Selected patients only	Most patients with advanced arthritis
Later surgery	Can be converted to a total knee if arthritis spreads	Revision if needed

Partial knee replacement is not automatically better than total knee replacement. The right operation depends on the pattern of your arthritis, which is confirmed through examination, X-rays and sometimes stress X-rays or MRI.

SEO	H2: “Partial Knee Replacement (UKR) in Nashik”.

NMC check
Name it “robotic partial knee replacement” only if partial knees are robotic-assisted at the institute (see the Robotic page note).

8 Recovery after knee replacement

#recovery · Typical timeline; sets realistic expectations.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Time after surgery	What usually happens
Day 0–1	Standing and first steps with a walker, supervised by a physiotherapist
Hospital stay (a few days)	Daily physiotherapy, bending exercises, walking in the corridor, stairs practice
Weeks 2–3	Wound check and stitch or clip removal; walking with a stick or walker at home
Weeks 4–6	Walking more independently; many people return to light household activities
Around 6 weeks	Driving may be possible once you can control the car safely, if your surgeon agrees
6–12 weeks	Return to desk work and most daily activities; longer for physical jobs
3–12 months	Continued improvement in strength and comfort; swelling settles gradually

Indian lifestyle questions

Many patients can sit on a chair comfortably and climb stairs after recovery. Sitting cross-legged may become possible for some, but deep squatting and kneeling put high stress on the implant and are usually discouraged. A western-style toilet or commode is advised, at least in the early weeks. Dr. Kasliwal will guide you on what is safe for your knee.

Recovery differs from person to person. These are typical ranges, not targets.

Design	Horizontal timeline (vertical on mobile). Link to /patient-guide#recovery for exercises.

NMC check
Keep timelines as ranges; no “walk the same day” guarantees.

9 Risks and complications

#risks · Required for balanced, NMC-compliant information.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Knee replacement is a common and generally safe operation, but like all surgery it carries risks. These include:

- Infection of the wound or the joint
- Blood clots in the leg veins (DVT), which can rarely travel to the lungs
- Stiffness or reduced bending
- Persistent pain or swelling in a small proportion of patients
- Wound-healing problems
- Rarely, injury to nearby nerves or blood vessels
- Loosening or wear of the implant over many years
- Risks related to anaesthesia and existing medical conditions

We reduce these risks with careful pre-operative assessment, a clean operating environment, antibiotics, blood-thinning medicines where appropriate, early mobilisation and close follow-up. Your individual risks are discussed before you consent.

10 Sports injuries & revision surgery

#related · Short cross-links; prevents duplicate content.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Ligament, meniscus and cartilage injuries are usually treated with keyhole surgery. See Knee & Shoulder Arthroscopy (/arthroscopy). If you already have a knee replacement that is painful or unstable, see Revision Joint Replacement (/robotic-joint-replacement#revision-joint-replacement).

Design	Three link cards: ACL & ligaments · Meniscus · Revision.

11 Cost & insurance

#cost-and-insurance · Replaces the cost page; captures “knee replacement cost Nashik”.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

The cost of knee replacement varies from patient to patient. We give you a written estimate after your consultation, once the plan is clear.

What affects the cost

Factor	Why it matters
Type of surgery	Total or partial; one knee or both; robotic-assisted or conventional
Implant	Different implant designs and materials have different prices
Room category	General, semi-private or private room
Length of stay	Depends on your recovery and medical needs
Tests and medical fitness	Pre-operative tests and any specialist opinions
Anaesthesia, medicines and physiotherapy	Included in the package or charged separately, as explained in your estimate

Insurance

Many health insurance policies cover knee replacement, including robotic-assisted surgery, subject to your policy terms. Our team helps with pre-authorisation and cashless processing with participating insurers and TPAs. Please bring your policy details to your consultation.

Payment options
Ask our billing team about available payment options.

SEO	H2: “Knee Replacement Cost in Nashik”. Do not publish fixed prices unless kept up to date.

NMC check

- Rename the menu item “Knee Surgery Cost & EMI” to “Cost & Insurance”. Do not advertise EMI, “0% interest”, discounts or packages as offers; the NMC prohibits inducements.
- Confirm the list of insurers/TPAs and cashless availability with the hospital.

12 Knee FAQs

#faqs · Accordion with FAQPage schema.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

At what age should knee replacement be done?
There is no fixed age. It is considered when arthritis seriously limits your life and other treatments have not helped. Most patients are over 55, but younger patients with severe damage may also benefit.

How long does a knee replacement last?
Most modern knee replacements continue to work well for 15 to 20 years or longer. Weight, activity and bone quality all play a part.

Is knee replacement very painful?
There is pain after surgery, but it is managed with nerve blocks, medicines and early movement. Most patients find it improves steadily over the first weeks.

How long will I stay in hospital?
Usually a few days for one knee. You go home once you can walk safely with support.

When can I walk after knee replacement?
Usually within a day of surgery, with a walker and a physiotherapist. Most people walk with a stick or independently within a few weeks.

Can I sit on the floor after knee replacement?
Some patients can sit cross-legged after recovery, but deep squatting and kneeling are generally discouraged. Ask your surgeon about your knee.

What is the difference between partial and total knee replacement?
A partial knee replaces only the worn part of the knee; a total knee replaces the whole joint surface. Suitability depends on your arthritis pattern.

Do I need surgery if my X-ray shows arthritis?
Not necessarily. Many people with arthritis on X-ray manage well with exercise and other treatment. Surgery depends on your symptoms and function.

Does insurance cover knee replacement?
Many policies do, subject to terms. Our team can help you check.

13 Closing CTA

#book ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Knee pain affecting your life? Book an assessment with Dr. Vishal Kasliwal to understand the cause and your options, from exercise and injections to partial or total knee replacement.

[Book a Consultation] [Call] [WhatsApp]

PAGE 3

Hip

Route	**/hip**
H1	**Hip Treatment & Hip Replacement in Nashik**
Title tag	Hip Replacement in Nashik | AVN & Hip Arthritis | Dr. Vishal Kasliwal
Meta description	Hip pain, hip arthritis, avascular necrosis (AVN) and total hip replacement in Nashik. Assessment and treatment by Dr. Vishal Kasliwal.
Primary keywords	hip replacement Nashik · hip replacement surgeon Nashik · AVN hip treatment Nashik
Secondary keywords	robotic hip replacement Nashik · hip arthritis treatment · avascular necrosis of hip · hip fracture surgery elderly Nashik · hip specialist Nashik
Schema (JSON-LD)	MedicalWebPage · MedicalCondition (AVN, hip OA) · MedicalProcedure (THR) · Physician · FAQPage · BreadcrumbList
Target length	3,000–4,000 words
Replaces old routes	/hip-specialist-nashik, /robotic-hip-replacement-nashik, /avn-hip-treatment-nashik



Demo layout (wireframe)

Numbers match the section numbers in the content pages that follow. T = sticky tabs.

1 Hero
H1, sub-line, buttons, real photo.

T Sticky tabs
Hip pain · Arthritis · AVN · Hip replacement · Recovery · Hip fracture · FAQs

2 Is it your hip or your back?
Body diagram illustration + copy.

3 Hip arthritis
Copy + non-surgical options.

4 AVN of the hip
Stage-by-stage treatment table.

5 Total hip replacement
Image + full procedure copy.

6 Recovery
Timeline.

7 Risks
List.

8 Hip fracture in the elderly
Callout with emergency number.

9 Revision
Link card.

10 FAQs

11 Closing CTA

Section-by-section content

1 Hero

#top ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

H1: Hip Treatment & Hip Replacement in Nashik

Assessment and treatment of hip pain, arthritis and avascular necrosis (AVN), including total hip replacement, by Dr. Vishal Kasliwal at Kasliwal Joint Institute, Nashik.

[Book a Consultation] [Call] [WhatsApp] · Credential line

2 Is the pain from your hip or your back?

#hip-pain · Common confusion; useful search content.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Hip joint problems usually cause pain in the groin or front of the thigh, sometimes spreading to the knee. Pain felt mainly in the buttock, or shooting down the back of the leg, is often from the lower back. Difficulty putting on socks or shoes, a limp, and stiffness when getting out of a car are typical of hip joint problems. An examination and X-rays help find the real source.

Design	Split: simple body diagram showing hip vs back pain areas (illustration).

3 Hip arthritis

#hip-arthritis ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Hip arthritis is wear of the cartilage in the ball-and-socket hip joint. Causes include age-related osteoarthritis, inflammatory arthritis such as rheumatoid arthritis or ankylosing spondylitis, earlier injury, childhood hip conditions, and avascular necrosis.

Symptoms

- Groin or thigh pain on walking
- Stiffness and difficulty bending to wear socks or cut toenails
- Limping, or one leg feeling shorter
- Difficulty sitting cross-legged or on the floor
- Pain at night

Non-surgical treatment

Early hip arthritis is managed with exercises, weight control, walking aids, activity changes and pain relief. Hip replacement is considered when pain and stiffness seriously limit daily life despite these measures.

4 Avascular necrosis (AVN) of the hip

#avn-hip · Replaces the AVN page; a high-interest topic in India.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Avascular necrosis (also called osteonecrosis) happens when the blood supply to the ball of the hip joint (femoral head) is reduced. The bone weakens and can collapse, leading to arthritis. It often affects adults between 30 and 50, and can affect both hips.

Common causes and risk factors

- Long-term or high-dose steroid medicines
- Heavy alcohol use
- Hip injury or fracture
- Sickle cell disease and some blood disorders
- Certain medical treatments and conditions; sometimes no cause is found

Early signs

Groin pain that starts gradually, worsens on walking and may be present at rest. X-rays can be normal in early AVN; MRI detects it earlier. Early diagnosis matters because some treatments work only before the bone collapses.

Stage	What it means	Treatment options
Early (before collapse)	Bone affected but the ball keeps its shape	Protected weight-bearing, medicines, and hip-preserving surgery such as core decompression, sometimes with bone grafting
Collapse	The ball begins to flatten	Hip preservation is less reliable; replacement is often considered
Arthritis	Joint surfaces damaged	Total hip replacement

Treatment depends on the stage, the size of the affected area, your age and symptoms. Dr. Kasliwal will explain which options suit you.

SEO	H2: “AVN (Avascular Necrosis) of the Hip: Treatment in Nashik”.

NMC check

Confirm which hip-preserving procedures Dr. Kasliwal performs before listing them.

5 Total hip replacement

#hip-replacement · Main procedure content; replaces the robotic hip page.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

In a total hip replacement, the damaged ball is replaced with a metal or ceramic head on a stem fixed into the thigh bone, and the socket is lined with a new cup. The implants may be fixed with bone cement or designed for the bone to grow onto them.

Technology-assisted planning

Careful planning of cup angle, leg length and hip offset helps the new hip feel stable and balanced. [Describe the system used: robotic or navigation-assisted, and its name.]

Step by step

- Assessment: examination, X-rays of the pelvis and hip, and a discussion of your goals
- Pre-operative tests and a medical fitness check
- Anaesthesia: usually spinal anaesthesia
- Surgery: usually about 1.5 to 2 hours
- Walking with support usually begins within a day, with physiotherapy
- Going home after a few days, once walking and stairs are safe

Leg length

Leg length is measured and planned before and during surgery. A small difference can sometimes remain; most people do not notice it, and a shoe insert helps if needed.

SEO	H2: “Hip Replacement in Nashik”. Add “robotic hip replacement” to copy only if the system is robotic.

NMC check

The current menu says “Robotic Hip Replacement (THR)” and “technology-assisted”. Confirm the system; if it is navigation, say “navigation-assisted” and change the menu label to “Hip Replacement (THR)”.

6 Recovery after hip replacement

#recovery ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Time after surgery	What usually happens
Day 0–1	Standing and walking with a walker, with a physiotherapist
Hospital stay (a few days)	Walking practice, stairs, safe movement and exercises
Weeks 2–6	Walking with a stick or independently; hip precautions as advised
Around 6 weeks	Driving and light activities may resume if your surgeon agrees
3 months onward	Most daily activities; continued strength gains up to a year

Depending on the surgical approach, you may be advised to avoid bending the hip too far, crossing your legs or low seats for the first weeks. Floor sitting and deep squatting should be discussed with your surgeon, as they can increase the risk of dislocation.

NMC check

Confirm the approach used (posterior or anterior) so precautions are accurate.

7 Risks

#risks ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Risks include infection, blood clots, dislocation, a difference in leg length, fracture of the bone during or after surgery, nerve injury (rare), loosening or wear over many years, and anaesthesia-related risks. These are explained in detail before surgery, together with the steps taken to reduce them.

8 Hip fracture in the elderly

#hip-fracture · Links to trauma care; time-sensitive information.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

A fall in an older person can break the top of the thigh bone (hip fracture). This is a serious injury: most patients need surgery, ideally within a day or two, so they can get out of bed quickly and avoid complications of lying in bed.
Depending on the fracture, treatment may be fixation with screws or a nail, a half hip replacement (hemiarthroplasty) or a total hip replacement.
For emergencies, call [emergency number].

Design	Sand callout with a click-to-call emergency button. Link to /arthroscopy#fracture-and-trauma-care.

NMC check
Confirm 24-hour emergency availability and the emergency number.

9 Revision hip surgery

#revision ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)
If you have a hip replacement that is painful, loose or dislocating, see Revision Joint Replacement (/robotic-joint-replacement#revision-joint-replacement).

10 Hip FAQs

#faqs ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

How long does a hip replacement last?
Most modern hip replacements continue to work well for 15 to 20 years or longer.

Can young patients with AVN have a hip replacement?
Yes, when the joint is badly damaged. Hip-preserving options are considered first if AVN is found early.

When can I walk after hip replacement?
Usually within a day of surgery, with support. Most people walk without aids within a few weeks.

Will my legs be the same length?
Leg length is carefully planned. A small difference can remain; most people do not notice it.

Can I sit on the floor after hip replacement?
This depends on the surgical approach and your recovery. Discuss it with your surgeon before trying.

Is hip pain always from arthritis?
No. It can come from the back, tendons, AVN or other causes. An examination and X-rays help find the cause.

11 Closing CTA

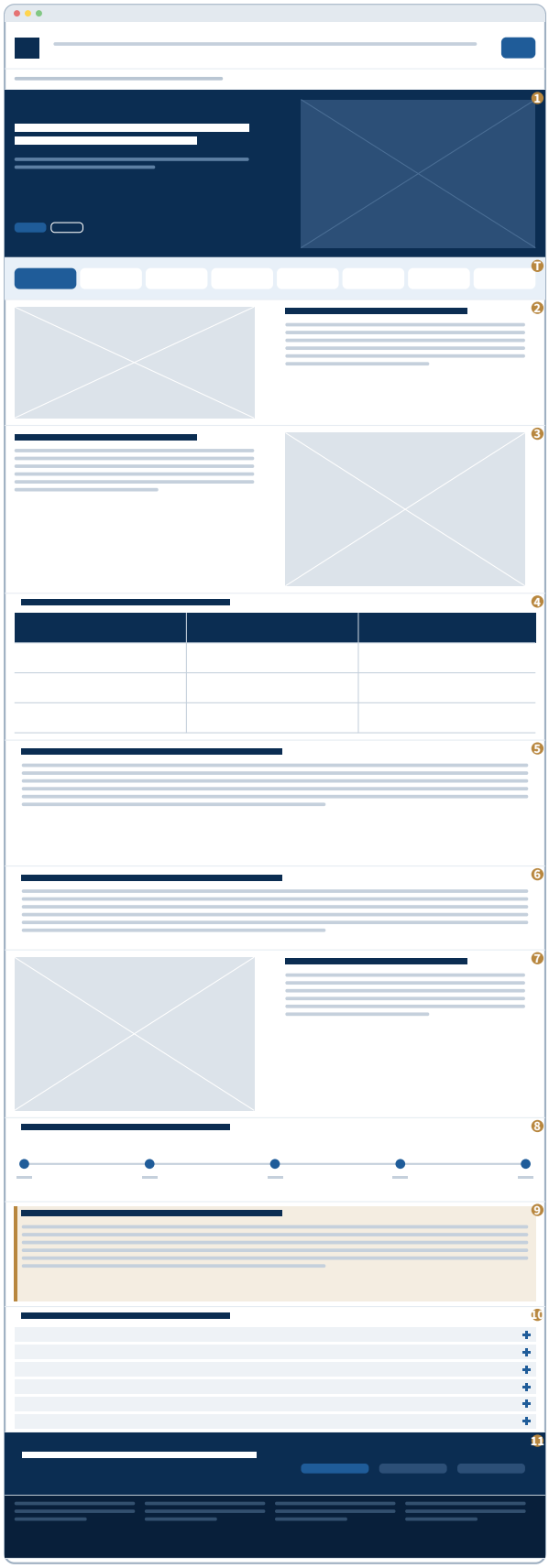
#book ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)
Hip pain, stiffness or a limp? Book an assessment with Dr. Vishal Kasliwal.
[Book a Consultation] [Call] [WhatsApp]

PAGE 4

Arthroscopy, Sports Injuries & Trauma

Route	**/arthroscopy**
H1	**Knee & Shoulder Arthroscopy in Nashik**
Title tag	Arthroscopy in Nashik | ACL, Meniscus & Shoulder Surgery | Dr. Vishal Kasliwal
Meta description	Keyhole surgery in Nashik for ACL tears, meniscus tears, PCL and cartilage injuries, kneecap instability, rotator cuff and shoulder dislocation, plus fracture care.
Primary keywords	knee arthroscopy Nashik · ACL surgery Nashik · shoulder arthroscopy Nashik
Secondary keywords	ACL reconstruction Nashik · meniscus tear treatment Nashik · rotator cuff repair Nashik · shoulder dislocation surgery · sports injury doctor Nashik · fracture treatment Nashik
Schema (JSON-LD)	MedicalWebPage · MedicalProcedure (arthroscopy, ACL reconstruction, rotator cuff repair) · Physician · FAQPage · BreadcrumbList
Target length	4,000–5,000 words
Replaces old routes	/knee-arthroscopy-nashik, /acl-reconstruction-nashik, /meniscus-tear-treatment-nashik, /pcl-cartilage-surgery-nashik, /shoulder-arthroscopy-nashik, /orthopaedic-trauma-care-nashik



Demo layout (wireframe)

Numbers match the section numbers in the content pages that follow. T = sticky tabs.

1 Hero
H1, sub-line, buttons, real arthroscopy theatre photo (no patient visible).

T Sticky tabs
Arthroscopy · ACL · Meniscus · PCL & cartilage · Kneecap · Shoulder · Trauma · FAQs

2 What is arthroscopy?
Illustration of keyhole portals + copy.

3 ACL reconstruction
Image + full copy with rehab phases.

4 Meniscus tear
Repair vs trimming table.

5 PCL & cartilage
Copy.

6 Kneecap instability
Copy.

7 Shoulder arthroscopy
Image + rotator cuff and instability copy.

8 Return to sport
Phase timeline.

9 Fracture & trauma care
Callout with emergency number.

10 FAQs

11 Closing CTA

Section-by-section content

1 Hero

#top ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

H1: Knee & Shoulder Arthroscopy in Nashik

Keyhole surgery for ligament, meniscus, cartilage and shoulder injuries, and orthopaedic fracture care, with Dr. Vishal Kasliwal, fellowship-trained in knee and shoulder arthroscopy.

[Book a Consultation] [Call] [WhatsApp] · Credential line

2 What is arthroscopy?

#what-is-arthroscopy ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Arthroscopy is keyhole surgery. Through two or three small cuts, the surgeon inserts a thin camera (arthroscope) and fine instruments into the joint. The inside of the joint is seen on a screen, and damaged tissue can be repaired, reconstructed or trimmed.

- Small incisions and less disturbance to muscles
- Usually a short hospital stay; some procedures are day-care
- Used both to diagnose problems and to treat them
- An MRI is usually done first to plan the procedure

Recovery depends on what is repaired. A simple trimming procedure needs a few weeks, while a ligament reconstruction needs months of rehabilitation.

SEO	H2: “Knee Arthroscopy in Nashik”.

3 ACL tear & ACL reconstruction

#acl-reconstruction ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

The anterior cruciate ligament (ACL) is one of the main ligaments that keep the knee stable. It is commonly torn during sports that involve twisting or sudden stops (football, kabaddi, cricket, badminton) and in falls from two-wheelers.

Symptoms

- A pop at the time of injury, followed by swelling
- The knee giving way, especially on turning or on stairs
- Difficulty returning to sport

Diagnosis

Diagnosis is made by examining the stability of the knee and confirmed with MRI, which also shows associated meniscus or cartilage damage.

Does every ACL tear need surgery?

No. People with less active lifestyles and a stable knee may do well with a structured rehabilitation programme. Reconstruction is usually advised for active people, those returning to sport, and when the knee keeps giving way, because repeated instability can damage the meniscus and cartilage.

ACL reconstruction

The torn ligament is replaced with a graft, usually a tendon taken from your own leg (hamstring, patellar or other tendon), fixed in place through keyhole surgery. Dr. Kasliwal will explain which graft suits you.

Rehabilitation

Walking with support usually starts within days, and knee bending is regained over the first weeks. Strength and balance training follow. Return to running is typically after a few months, and return to competitive sport usually takes around 9 months or more, once strength and stability tests are satisfactory.

SEO	H2: “ACL Reconstruction Surgery in Nashik”.

NMC check

Confirm graft choices Dr. Kasliwal commonly uses before naming specific ones.

4 Meniscus tear

#meniscus-tear ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

The menisci are two C-shaped cushions that act as shock absorbers between the thigh and shin bones. They can tear with a twisting injury in younger people, or gradually with age.

Symptoms

- Pain on the inner or outer side of the knee
- Swelling
- Catching or clicking
- Locking: the knee getting stuck and not straightening fully

	Meniscus repair	Partial meniscectomy (trimming)
What is done	Torn edges are stitched together	Only the unstable torn part is removed
Best for	Tears in areas with good blood supply, younger patients, root tears	Tears that cannot heal
Recovery	Longer; protected movement for several weeks	Quicker; usually a few weeks
Aim	Preserve the meniscus to protect the knee long-term	Relieve mechanical symptoms

Wherever possible, the meniscus is preserved, because losing it increases the risk of arthritis later. Many degenerative tears in older adults are treated without surgery, with exercise and physiotherapy.

SEO	H2: “Meniscus Tear Treatment in Nashik”.

5 PCL & cartilage injuries

#pcl-and-cartilage ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

PCL injuries

The posterior cruciate ligament (PCL) is often injured when the knee hits a dashboard or in a fall on a bent knee. Many isolated PCL injuries heal well with a brace and rehabilitation. Reconstruction is considered when the knee remains unstable or other ligaments are also injured.

Cartilage damage

Injury can damage a localised area of the smooth joint cartilage, causing pain, swelling and catching. Treatment depends on the size and location of the damage and your age, and may include rehabilitation, arthroscopic smoothing, techniques that stimulate new repair tissue, or cartilage transfer procedures.

NMC check
List only cartilage procedures Dr. Kasliwal performs.

6 Kneecap (patellar) instability

#kneecap-instability ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)
Some people, often teenagers and young adults, have a kneecap that slips out of place (dislocates). A first dislocation is usually treated with rehabilitation. If it keeps happening, surgery such as reconstruction of the MPFL ligament, which holds the kneecap in place, can stabilise it.

7 Shoulder arthroscopy

#shoulder-arthroscopy ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Rotator cuff tears

The rotator cuff is a group of tendons that lift and rotate the arm. Tears are common after 40, either from wear or an injury. Symptoms include pain when lifting the arm, night pain and weakness. Many tears respond to physiotherapy and injections; larger or painful tears that do not improve may be repaired with keyhole surgery.

Shoulder dislocation and instability (Bankart repair)

After a shoulder dislocates, the labrum (the rim of the socket) can tear, and the shoulder may keep dislocating, especially in young, active people. An arthroscopic Bankart repair reattaches the labrum to stabilise the shoulder.

Other shoulder conditions

- Impingement and bursitis: pain on overhead activity
- Frozen shoulder: painful stiffness, usually treated without surgery
- Labral and biceps tendon problems

Recovery

After shoulder repair, the arm is usually rested in a sling for a few weeks, followed by staged physiotherapy. Full recovery often takes 4 to 6 months.

Dr. Kasliwal completed advanced shoulder arthroscopy training in South Korea.

SEO	H2: “Shoulder Arthroscopy in Nashik”.

8 Returning to sport

#return-to-sport ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)
Rehabilitation is as important as surgery. It progresses in stages: controlling pain and swelling, regaining movement, rebuilding strength and balance, then sport-specific training. Return to sport is based on strength and function tests, not only on time.

Design	Five-phase horizontal timeline.

9 Fracture & trauma care

#fracture-and-trauma-care · Kept short so the site stays focused on joints.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Kasliwal Super Speciality Hospital provides orthopaedic care for fractures and injuries, including:

- Fractures of the arm, leg, wrist and ankle
- Hip fractures in older people (see /hip#hip-fracture)
- Complex and multiple injuries, with intensive care support
- Ligament and joint injuries from accidents

For emergencies, call [emergency number].

Design	Sand callout with click-to-call emergency button.

NMC check

Confirm emergency hours, ICU availability and the emergency number.

10 Arthroscopy FAQs

#faqs ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Is arthroscopy a major operation?

It is done through small cuts, but it is still surgery under anaesthesia. Recovery depends on what is repaired.

How long is the hospital stay?

Often one night or day-care for simple procedures; slightly longer after reconstructions.

When can I return to sport after ACL surgery?

Usually around 9 months or more, once strength and stability tests are satisfactory.

Can a torn meniscus heal on its own?

Small tears in the outer edge, and many age-related tears, can settle without surgery. Locking tears usually need treatment.

Do all shoulder dislocations need surgery?

No. Surgery is considered when the shoulder keeps dislocating or the risk of recurrence is high.

Do I need an MRI before arthroscopy?

Usually yes, to confirm the diagnosis and plan the procedure.

11 Closing CTA

#book ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

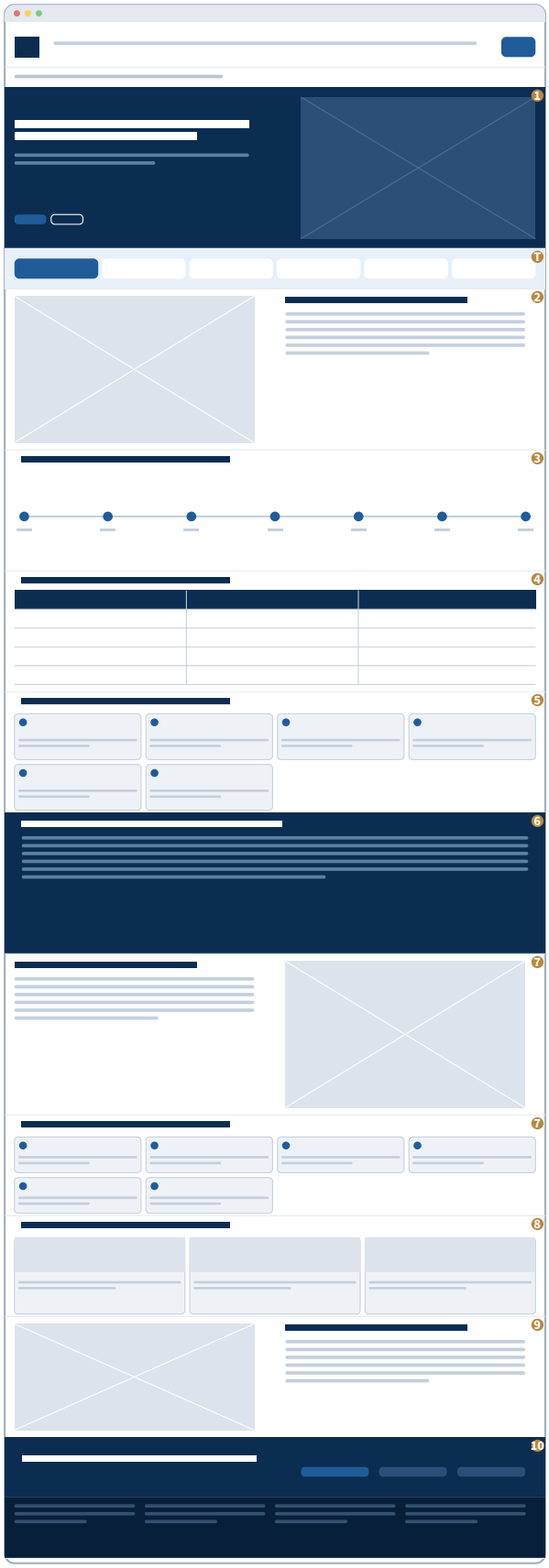
Injured your knee or shoulder? Book an assessment with Dr. Vishal Kasliwal.

[Book a Consultation] [Call] [WhatsApp]

PAGE 5

About

Route	**/about**
H1	**About Dr. Vishal Kasliwal & Kasliwal Joint Institute**
Title tag	Dr. Vishal Kasliwal | Joint Replacement Surgeon, Nashik | Kasliwal Joint Institute
Meta description	Dr. Vishal Kasliwal, M.S. Ortho, joint replacement and arthroscopy surgeon: training, surgical philosophy and Kasliwal Super Speciality Hospital, Gangapur Road, Nashik.
Primary keywords	Dr Vishal Kasliwal · joint replacement surgeon Nashik · Kasliwal Hospital Nashik
Secondary keywords	orthopaedic surgeon Nashik · Kasliwal Super Speciality Hospital · orthopaedic hospital Gangapur Road Nashik
Schema (JSON-LD)	AboutPage · Physician (with credentials, sameAs links to directory profiles) · Hospital/MedicalClinic · BreadcrumbList
Target length	1,800–2,500 words
Replaces old routes	/dr-vishal-kasliwal, /surgical-philosophy, /hospital, /patient-stories (removed)



Demo layout (wireframe)

Numbers match the section numbers in the content pages that follow. T = sticky tabs.

1 Hero
H1 + intro; portrait of Dr. Kasliwal.

T Sticky tabs
Dr. Kasliwal · Training · Philosophy · Hospital · Team · Visit us

2 Meet Dr. Vishal Kasliwal
Portrait + bio + credential and registration line.

3 Education & training
Vertical timeline.

4 Career
Table of positions.

5 Areas of clinical focus
Icon grid.

6 Surgical philosophy
Navy panel (homepage style).

7 The hospital
Exterior photo + history, then facilities as an icon list.

8 Care team
Team photos (real staff).

9 Visit us
Map + address + OPD timings.

10 Closing CTA

Section-by-section content

Sources for this page

Facts below are taken from Dr. Kasliwal's earlier websites (kasliwalhospital.in / .com, kasliwalknee clinic, kasliwalortho.com), his YouTube channel and LinkedIn profile, and directory profiles (HexaHealth, ClinicSpots, Lybrate, Timesmed). They are not always consistent, so every item marked [confirm] must be checked with Dr. Kasliwal and his CV before publishing.

1 Hero

#top ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

H1: About Dr. Vishal Kasliwal & Kasliwal Joint Institute

Kasliwal Joint Institute is the joint replacement and arthroscopy centre of Kasliwal Super Speciality Hospital, Nashik, led by orthopaedic surgeon Dr. Vishal Kasliwal. We focus on knee, hip and shoulder care, from accurate diagnosis and non-surgical treatment to robotic-assisted joint replacement and keyhole surgery.

2 Meet Dr. Vishal Kasliwal

#dr-vishal-kasliwal · Replaces the old doctor page.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Dr. Vishal Kasliwal

M.S. Orthopaedics · F.I.J.R. (Hip & Knee) · F.I.A. (Shoulder), South Korea
Joint Replacement & Arthroscopy Surgeon · Director, Kasliwal Super Speciality Hospital, Nashik
Registration: [Maharashtra Medical Council / NMR number] [confirm]

Dr. Vishal Kasliwal is an orthopaedic surgeon in Nashik whose practice is focused on joint replacement of the knee and hip and arthroscopic (keyhole) surgery of the knee and shoulder. He trained in computer-navigated joint replacement at Breach Candy Hospital, Mumbai, under Dr. Arun Mullaji, completed an arthroscopy fellowship in Pune under Dr. Sachin Tapasvi, and trained in shoulder arthroscopy in South Korea under Dr. Jin Young Park.

He performs robotic-assisted knee replacement with the VELYS™ system, alongside partial knee replacement, hip replacement, revision surgery and arthroscopic ligament and shoulder procedures. In [year] he has [approximately 5,000] joint replacement and arthroscopy procedures to his name [confirm figure and wording from records].

He has also taught orthopaedics as a lecturer at NDMVP Medical College, Nashik, and [has trained fellow surgeons in VELYS™ robotic knee replacement] [confirm].

Design	Split: professional portrait (real photo) left; copy right; credential line in a navy strip.

NMC check

- The NMC guidelines require the doctor's name, recognised qualifications, registration status and SMR/NMR number on doctor-related communication. Add the registration number here and in the footer.
- Do not use “best”, “leading”, “successful surgeries” or “one of the few”, which appear on his older sites. Use the procedure count only if records support it.
- Experience figures online vary (8, 10, 14, 15, 21 years). Use one confirmed figure.

3 Education & training

#training ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Year	Qualification / training	Institution
2005	MBBS	Mahadevappa Rampure Medical College, Kalaburagi (Gulbarga), Karnataka
2010	M.S. Orthopaedics	Jawaharlal Nehru Medical College, Belagavi (Belgaum), Karnataka
2011	Fellowship in Computer-Navigated Joint Replacement (Arthroplasty)	Breach Candy Hospital, Mumbai, under Dr. Arun Mullaji
2014	Fellowship in Knee & Shoulder Arthroscopy	Pune, under Dr. Sachin Tapasvi
2015	Fellowship in Shoulder & Elbow Arthroscopy	Neon Orthopaedic Clinic/Hospital, Seoul, South Korea, under Dr. Jin Young Park [confirm name]

Design	Vertical timeline with years on the left.

NMC check

Confirm years, exact fellowship titles and the Korean institution's name. Directory listings vary.

4 Career

#career ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Period	Position
2010–2012	Senior Registrar, Orthopaedics, K.E.M. Hospital, Mumbai
2012–2013	Consultant, Mewar Group of Orthopaedic Hospitals, Udaipur
2015–2016	Lecturer, Orthopaedics, NDMVP Medical College, Nashik
Present	Director, Kasliwal Super Speciality Hospital, Nashik; leads Kasliwal Joint Institute
[confirm]	Other current hospital affiliations (directories also list Vijan Hospital and earlier Wockhardt Hospitals, Nashik)

NMC check

One directory lists “Registrar, YCM Hospital, 2005–2010”, which overlaps with his MS training. Confirm before including.

5 Areas of clinical focus

#expertise ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

- Robotic-assisted (VELYS™) total knee replacement
- Partial knee replacement
- Total hip replacement
- Revision knee and hip replacement
- Arthroscopic ACL and ligament reconstruction
- Meniscus repair
- Shoulder arthroscopy: rotator cuff repair and stabilisation
- Fracture and trauma care

Design	Icon grid; each item links to its page anchor.

6 Our surgical philosophy

#surgical-philosophy · Replaces the old philosophy page.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Not every knee is the same. So why should every knee be replaced the same way?

- **Personalised alignment**: the surgical plan is based on your anatomy and ligament balance rather than one fixed target for every knee.
- **Precision**: robotic assistance helps translate the plan into accurate bone preparation and implant position.
- **Preserving what matters**: where appropriate, muscle-sparing (subvastus) approaches and preservation of healthy structures such as the posterior cruciate ligament.
- **The right operation, or none**: non-surgical care first; partial replacement when suitable; total replacement when needed.
- **Functional recovery**: early mobilisation and a structured physiotherapy programme.

Design	Navy panel, same style as the homepage philosophy block.

7 Kasliwal Super Speciality Hospital

#hospital · Replaces the old hospital page.

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Kasliwal Hospital was established in Nashik in 1981. Its super speciality unit at Bosco Centre, Prasad Circle, Gangapur Road, is home to Kasliwal Joint Institute, with facilities designed for joint replacement and orthopaedic surgery.

Facilities [confirm each]

- Operation theatres for joint replacement, with laminar airflow
- VELYS™ robotic-assisted surgery system
- Intensive care unit with round-the-clock doctor cover
- In-house physiotherapy and rehabilitation
- Inpatient rooms in different categories
- Pharmacy and diagnostic support
- Cashless insurance processing with participating insurers

Other departments

[List other hospital departments, e.g. paediatrics and obstetrics & gynaecology, if they should appear here.]

Design	Split: real exterior photo + copy; facilities as an icon list.

NMC check

- State facilities as facts only (no “most advanced”, “Nashik's only”). Older sites claimed “Nashik's only centre…” and “painless”; do not reuse.
- Confirm the official hospital name spelling (Super Speciality / Superspeciality).

8 Our care team

#team ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Your care is shared by a team: orthopaedic surgeons, anaesthetists, physicians, physiotherapists, nursing staff and patient coordinators who help with appointments, insurance and discharge. [Add names, qualifications and photos of key team members.]

Design	Cards with real staff photos, name, role and qualifications.

NMC check

Include registration numbers for any doctors named.

9 Visit us

#visit-us ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Kasliwal Super Speciality Hospital
1st Floor, Bosco Centre, Prasad Circle, Gangapur Road, Nashik, Maharashtra 422005
Phone: [confirmed number] · Email: kasliwalsuperspeciality.hosp@gmail.com
OPD: Mon–Sat, 10:00 AM–2:00 PM & 5:00 PM–8:00 PM [confirm]
Landmarks: Prasad Circle, Gangapur Road. [Add parking information.]

Design	Embedded Google Map (link to the actual Business Profile) + address card.
SEO	Address must match the Google Business Profile and all directories exactly.

10 Closing CTA

#book ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

Book a consultation with Dr. Vishal Kasliwal at Kasliwal Super Speciality Hospital, Nashik.
[Book a Consultation] [Call] [WhatsApp]

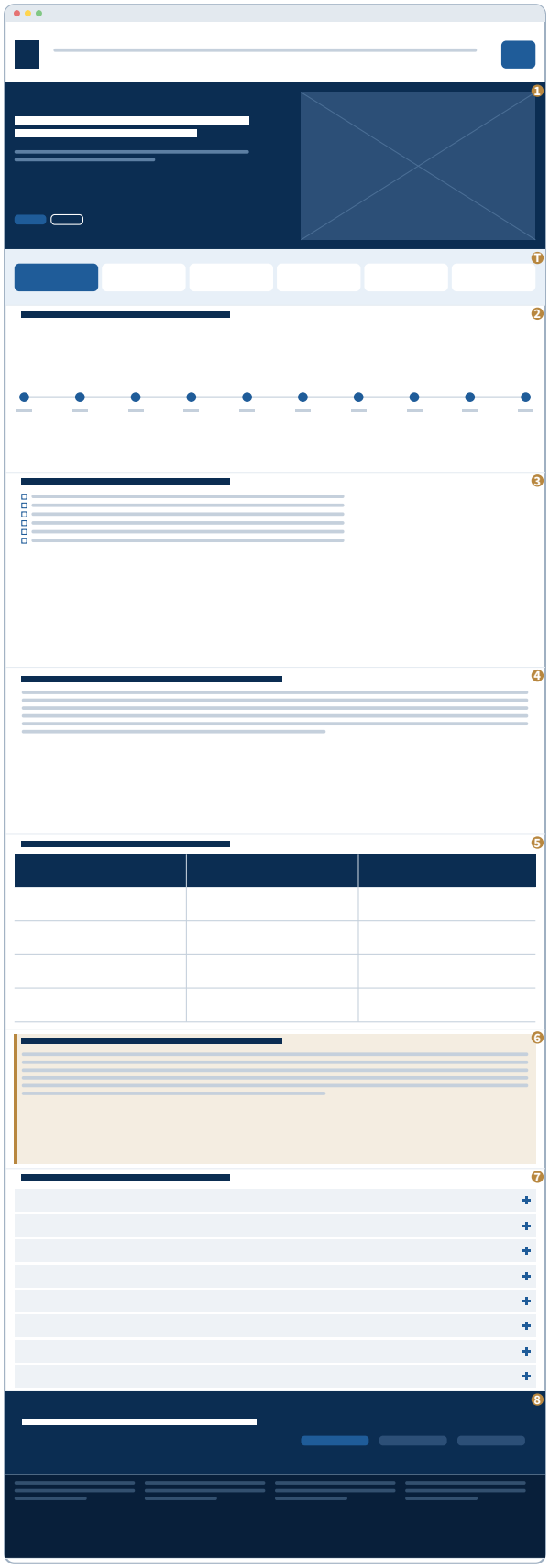
Removed from this page

Patient Stories, testimonials, ratings and review widgets are not included, as required by the NMC guidelines.

PAGE 6

Patient Guide

Route	**/patient-guide**
H1	**Patient Guide: Your Joint Replacement Journey**
Title tag	Patient Guide | Preparing for & Recovering from Joint Surgery | Kasliwal Joint Institute
Meta description	What to expect before, during and after joint replacement or arthroscopy at Kasliwal Joint Institute, Nashik, including help for patients travelling from outside Nashik.
Primary keywords	knee replacement recovery · preparing for knee replacement
Secondary keywords	what to expect after knee replacement · hospital stay knee replacement · joint replacement Nashik outstation patients
Schema (JSON-LD)	MedicalWebPage · FAQPage · BreadcrumbList
Target length	2,000–3,000 words
Replaces old routes	/patient-guide/* sub-pages



Demo layout (wireframe)

Numbers match the section numbers in the content pages that follow. T = sticky tabs.

1 Hero
H1 + sub-line.

T Sticky tabs
Journey · Preparing · In hospital · Recovery · Outstation · FAQs

2 Your surgery journey
10-step timeline.

3 Preparing for surgery
Checklist + downloadable PDF.

4 In hospital
Copy.

5 Recovery & exercises
Table + exercise videos.

6 Outstation patients
Callout.

7 General FAQs
All site-wide FAQs.

8 Closing CTA

Section-by-section content

1 Hero

#top ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

H1: Patient Guide: Your Joint Replacement Journey

Everything you need to know before, during and after your surgery at Kasliwal Joint Institute, so you know what happens next at every step.

[Book a Consultation] [Call] [WhatsApp]

2 Your surgery journey

#journey ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

- **1 Consultation**: history, examination and your goals
- **2 Diagnosis**: X-rays and tests as needed
- **3 Treatment decision**: non-surgical options first; surgery only when appropriate
- **4 Planning**: pre-operative tests, fitness check and the surgical plan
- **5 Admission & surgery**
- **6 Early mobilisation**: walking with support soon after surgery
- **7 Physiotherapy**: supervised exercises in hospital
- **8 Going home**: medicines, dressing care and a home exercise plan
- **9 Follow-up**: wound review and exercise progression
- **10 Back to movement**: a gradual return to daily life

3 Preparing for surgery

#preparing ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)

- Bring all reports, X-rays and a list of your medicines
- Control blood sugar and blood pressure; tell us about blood thinners
- Treat any dental, skin or urine infection before surgery
- Arrange a western toilet or commode, a firm chair with arms and a clear walking path at home
- Practise the exercises your physiotherapist shows you
- Arrange a family member to stay with you for the first weeks

Download: Pre-surgery checklist (PDF).

4 Your hospital stay

#in-hospital ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)
You are usually admitted the day before or the morning of surgery. After the operation you are monitored closely, pain is controlled with nerve blocks and medicines, and physiotherapy begins early. Before discharge, you learn safe walking, stairs and your home exercises, and receive written instructions.

5 Recovery & rehabilitation

#recovery ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)
Recovery timelines are given on the Knee (/knee#recovery) and Hip (/hip#recovery) pages. Here, add exercise videos by the physiotherapy team, wound-care guidance and warning signs that need a call to the hospital: fever, increasing redness or discharge from the wound, calf pain or swelling, and sudden breathlessness (an emergency).

6 Patients from outside Nashik

#outstation-patients ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)
Many patients travel from across North Maharashtra. You can share your X-rays and reports on WhatsApp before your visit so the team can advise which tests to bring. We can help you plan the number of days to stay, nearby accommodation and follow-up visits; some follow-up reviews can be done by video call.

NMC check
Avoid wording such as “fast-track” or travel offers that could read as inducements.

7 Frequently asked questions

#faqs ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)
Collect general questions here (appointments, documents, insurance, visitors, food, follow-up). Procedure-specific FAQs stay on their own pages.

8 Closing CTA

#book ·

DRAFT COPY (READY TO USE, PENDING DOCTOR'S REVIEW)
Have a question about your treatment? Our team is here to help.
[Book a Consultation] [Call] [WhatsApp]

BEFORE PUBLISHING

NMC compliance checklist for every page

Check	Status needed
No superlatives: best, leading, only, first, No. 1, painless, guaranteed	Removed from all copy and alt text
No testimonials, reviews, ratings, before/after images	Removed site-wide, including the homepage widget and About menu
No AI-generated images of patients, surgery on patients or outcomes	Replace with real photos or clear illustrations
Registration number with the doctor's name	Footer, About page and credential lines
Numbers (years, procedures) supported by records	Confirmed in writing by Dr. Kasliwal
Benefits written as evidence-based possibilities, with risks shown	Risks sections present on Knee and Hip pages
No discounts, offers or promotional EMI	Cost section factual only
Medical review by Dr. Kasliwal	Byline and date on each page
No paid patient referrals or pay-per-patient agency deals	Applies to ads and lead generation too

Facts to confirm with the client

#	Item
1	Registration number (MMC/NMR) and exact designation
2	Final experience figure and procedure count, with wording
3	VELYS™ details: implant used (ATTUNE®), step order, whether partial knees and hips are robotic or navigation-assisted
4	Revision surgery, hip-preserving AVN surgery, cartilage procedures and graft choices he performs
5	Education and career dates, fellowship titles and the Korean institution's name
6	Hospital facilities (laminar airflow OT, ICU cover), departments, emergency number and hours
7	Typical operation time, length of stay and anaesthesia used
8	Primary phone number, official hospital name spelling, OPD timings
9	Insurers/TPAs and cashless availability
10	Real photos: portrait, OT, VELYS™, hospital exterior and interior, team

References for evidence statements

Morrison SR, Hall AJ, Clement ND, et al. Use of the VELYS Robotic-Assisted Solution in knee arthroplasty: a scoping review. Journal of Experimental Orthopaedics, 2026; 13(2): e70726. Reports improved implant positioning and a non-inferior safety profile, and notes that the evidence is limited and of moderate-to-poor quality.

NMC Guidelines on Ethical Advertising and Public Communication by Hospitals/Medical Institutions and Registered Medical Practitioners, public notice dated 6 October 2026 (in force immediately).